

July 20, 2026 04:05 AM GMT

Erasca, Inc. | North America

'0015 Update: Path Clarifies; '4001 Next Catalyst

WHAT'S CHANGED

Erasca, Inc. (ERAS.O)	From	To
Price Target	\$12.00	\$21.00

We raise our price target following the recent ERAS-0015 updates reflecting a strengthening profile and clearer path to registration, but remain Equal-weight as we await more mature and durable data. ERAS appears well positioned within a competitive RAS landscape.

Incremental progress, but awaiting further validation. Recent updates improve visibility into a differentiated, multi-indication registration opportunity, but we remain Equal-weight pending more mature data and longer follow-up. The initial ERAS-0015 update highlighted potential best-in-class attributes, though the thesis remains anchored to a small, early Phase 1 dataset, an emerging combination signal, and an evolving path from expansion into registration. The latest data begin to address these elements. Efficacy has been sustained and, in some cases, improved with additional patients and follow-up, early combination feasibility has been demonstrated at standard doses, and mgmt. has outlined a clearer and more accelerated path into registration-enabling studies. Upcoming readouts, including ERAS-4001 preliminary Phase 1 monotherapy data in 2H26 and '0015 monotherapy expansion and combination dose-escalation data, including panitumumab, in 1H27, should be important for further validation. Taken together, ERAS appears well positioned as a funded, multi-indication registration story, but we believe additional data and follow-up will remain key.

The recent update (deck [here](#)) strengthens the PDAC efficacy profile, though durability remains immature. The data provide a consistent cut-to-cut comparison across a US-only 2L+ population using a ≥8-week efficacy-evaluable definition (April vs. May data cutoffs). uORR increased from 14% to 25% at 24mg, from 50% to 57% at 32mg, and from 27% to 40% across pooled 24-32mg RDEs. In the US 2L+ KRAS G12X PDAC subset, uORR (≥8 weeks) at 32mg was 57% (4/7), up from 50% (2/4), as the cohort expanded from four to seven evaluable patients, with responses increasing from two to four. Across pooled 24-32mg RDEs, activity improved to 40% (6/15) from 27% (3/11), with all responders (confirmed and unconfirmed) remaining on treatment. A dose-response trend remains evident (11%/0%/25%/57% across 8/16/24/32mg), and PK data show dose-dependent exposure without an observed plateau. *That said, the 57% uORR at 32mg is based on a small sample size with limited durability follow-up, and will require confirmation through greater consistency and depth of response with additional patients and longer follow-up.*

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Erasca, Inc. (ERAS.O, ERAS US)

Biotechnology | United States of America

Stock Rating	Equal-weight
Industry View	Attractive
Price target	\$21.00
Shr price, close (Jul 16, 2026)	\$19.02
Mkt cap, curr (mm)	\$5,788
52-Week Range	\$24.28-1.33

Fiscal Year Ending	12/25	12/26e	12/27e	12/28e
EPS (\$) **	(0.44)	(0.90)	(0.55)	(0.66)
Prior EPS (\$) **	-	(0.94)	(0.49)	(0.56)
P/E	NM	NM	NM	NM
EPS (\$) §	(0.44)	(0.95)	(0.56)	(0.66)
Div yld (%)	-	-	-	-

Unless otherwise noted, all metrics are based on Morgan Stanley ModelWare framework

** = Based on consensus methodology

§ = Consensus data is provided by Refinitiv Estimates

e = Morgan Stanley Research estimates

QUARTERLY EPS (\$)

Quarter	2025	2026e Prior	2026e Current	2027e Prior	2027e Current
Q1	(0.11)	-	(0.60)a	(0.12)	(0.13)
Q2	(0.12)	(0.11)	(0.11)	(0.12)	(0.13)
Q3	(0.11)	(0.12)	(0.11)	(0.13)	(0.14)
Q4	(0.10)	(0.12)	(0.11)	(0.13)	(0.15)

e = Morgan Stanley Research estimates, a = Actual Company reported data

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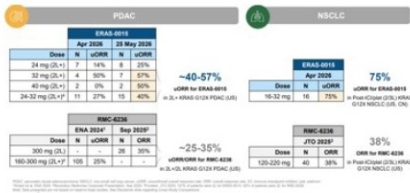
Exhibit 1: Efficacy data for 2L+ KRAS G12X PDAC pts across April and May data cutoffs

Patients who received first dose of ERAS-0015 at least 8 weeks prior to Apr 2028 DCO					
ERAS-0015 2L+ KRAS G12X PDAC	8 mg QD 2w/3	16 mg QD 2w/3	24 mg QD 2w/3	32 mg QD 2w/3	40 mg QD 2w/3
uORR, n (%)	1 (11)	0	1 (14)	2 (20)	0
DCR, n (%)	4 (44)	8 (100)	5 (71)	4 (100)	2 (100)

Patients who received first dose of ERAS-0015 at least 8 weeks prior to May 25, 2028 DCO					
ERAS-0015 2L+ KRAS G12X PDAC	8 mg QD 2w/3	16 mg QD 2w/3	24 mg QD 2w/3	32 mg QD 2w/3	40 mg QD 2w/3
uORR, n (%)	1 (11)	0	2 (28)	4 (37)	1 (10)
DCR, n (%)	4 (44)	8 (100)	6 (75)	7 (100)	2 (100)

Source: Company reports

Exhibit 3: ERAS-0015 efficacy vs. comparator pan-RAS molecular glue RMC-6236 benchmark



Source: Company reports

Exhibit 2: Updated time on treatment for 2L+ KRAS G12X PDAC pts



Source: Company reports

Exhibit 4: ERAS-0015 safety vs. comparator pan-RAS molecular glue RMC-6236 benchmark

ERAS-0015	RMC-6236			
	Ph 1 16-32 mg QD 1w/1	Ph 1 160-300 mg QD 1w/1	Ph 1 1800 mg QD 1w/1	Ph 1 1800 mg QD 1w/1
Rash	81%	75%	81%	81%
Diarrhea	32%	32%	32%	32%
Stomatitis	19%	19%	19%	19%
Nausea	13%	13%	13%	13%
Dose interruptions	13%	13%	13%	13%
Dose reductions	8%	8%	8%	8%
Dose discontinuations	0%	0%	0%	0%

Source: Company reports

Safety profile supports sustained dosing and combination potential. At 16-32mg, the safety database has expanded to N=72 (from 4-3 previously). Rash occurred in 72% of patients (including 3% Gr3), diarrhea in 32% (1% Gr3), and stomatitis in 19% (3% Gr3). Dose interruptions occurred in 13% of patients, reductions in 8%, and there were no TRAE-related discontinuations. There is no clear dose gradient in common toxicities, with rash observed in 81%/75%/64% of patients at 16/24/32mg, respectively. Median relative dose intensity (RDI) was 100% across all dose levels, with mean RDI of 95.0% at 24-mg, 93.5% at 32mg, and 95.9% pooled. Cross-study comparisons against daraxonrasib (RMC-6236) suggest lower rates of Gr3+ rash and stomatitis and fewer dose modifications for '0015, though the comparison spans different trial phases and populations and '0015 follow-up remains less mature. Notably, each program has reported a single Gr5 pneumonitis event: '0015 in AURORAS-1 (a Gr3 event that progressed to Gr5 following patient-elected withdrawal of supportive care) and daraxonrasib in the Ph3 RASolute 302 trial (0.4%). We view this as a possible class-related risk rather than a clearly molecule-specific signal.

Exhibit 5: Updated ERAS-0015 safety summary by grade

Summary of TRAEs occurring in ≥10% of patients					
TRAEs, n (%)	Grade 1	Grade 2	Grade 3	Grade 4	All Grades
Rash, n (%)	37 (51)	13 (18)	2 (3)	0	52 (72)
Diarrhea, n (%)	17 (24)	5 (7)	1 (1)	0	23 (32)
Stomatitis, n (%)	9 (12)	3 (4)	2 (3)	0	14 (19)
Nausea, n (%)	9 (12)	1 (1)	0	0	10 (14)
TRAEs leading to dose interruptions, n (%)	9 (12)	0	0	0	9 (12)
TRAEs leading to dose reductions, n (%)	0	0	0	0	0
TRAEs leading to dose discontinuations, n (%)	0	0	0	0	0

Source: Company reports

Exhibit 6: Updated ERAS-0015 safety summary by dose

Summary of TRAEs occurring in ≥10% of patients				
TRAEs, n (%)	16 mg QD (24-16)	24 mg QD (24-40)	32 mg QD (24-40)	16-32 mg (24-16)
Rash, n (%)	13 (51)	21 (75)	18 (64)	52 (72)
Diarrhea, n (%)	5 (21)	11 (38)	7 (25)	23 (32)
Stomatitis, n (%)	4 (25)	5 (18)	5 (18)	14 (19)
Nausea, n (%)	3 (12)	3 (11)	4 (14)	10 (14)
TRAEs leading to dose interruptions, n (%)	1 (5)	6 (21)	2 (7)	9 (12)
TRAEs leading to dose reductions, n (%)	1 (5)	2 (7)	3 (11)	6 (8)
TRAEs leading to dose discontinuations, n (%)	0	0	0	0

Source: Company reports

Combination feasibility emerging as a key point of differentiation... The recent update further supports the combination thesis, particularly around tolerability. As

of the July 6 data cut, '0015 16mg + standard-dose panitumumab cleared its initial dose level with no DLTs in four DLT-evaluable patients. Backfill at 16mg and escalation to 24mg are ongoing. *While there is no incremental efficacy disclosure beyond the prior uPR, we view this as representing an important step given the historical tolerability challenges associated with RAS combination strategies.*

...and beyond panitumumab, the combination opportunity continues to expand.

Merck (covered by Terence Flynn) is supplying pembrolizumab as part of the Erasca-sponsored AURORAS-1 study, while Tango will sponsor a Phase 1/2 study of '0015 + PRMT5i vopimetostat in MTAP-deleted pancreatic cancer and RASm NSCLC. We view these collaborations as early signs of potential as the monotherapy dataset matures. *While EGFR, PD-1, and PRMT5 combinations suggest the possibility for '0015 to function as a combinable RAS backbone, we view upcoming combination readouts as essential to establish the balance of efficacy, tolerability, dose intensity, and combinability, and ultimately to support a more differentiated positioning.*

Clearer path, but contingent on alignment and execution. In addition to the data to date, increased clarity on the registration strategy supports the outlook.

The company plans to initiate a potentially registration-enabling 2L+ NSCLC study in 1H27, followed by a 1L PDAC Phase 3 in 2027 and a RASm NSCLC pivotal Phase 3 in 2H27-1H28. If executed, this would move '0015 from early clinical development into three registration-directed programs within ~18-24 months. The sequencing appears logical, with NSCLC leading via a cleaner monotherapy path, while PDAC moves earlier-line where tolerability and combination flexibility may be more impactful. The company also announced an upsized public offering totaling \$632.5mn in gross proceeds, which should support parallel registrational development. That said, execution remains dependent on regulatory alignment and timely trial initiation, particularly as the first NSCLC study is expected to begin within a similar window as upcoming data updates. *Taken together, the outlined path is defined and funded, but we acknowledge still requires continued regulatory clarity and operational execution.*

IP overhang persists, with a data-dependent path to mitigation. On IP, we continue to see a credible path for the current valuation discount to diminish if clinical differentiation is sustained, though we do not view the risk as resolved at this stage (see our published analysis assessing the patent infringement risk [here](#)). Revolution Medicines' (RVMD, not covered) claim relies on the Doctrine of Equivalents rather than literal infringement, and our prior analysis suggests limited precedent for successful claims under this framework, albeit with small sample sizes. More importantly, sustained differentiation across dose, PK/PD, efficacy, safety, dose intensity, and combinability could increasingly support arguments around function-way-result and insubstantial differences. That said, this will require a more mature and clearly differentiated clinical profile over time. *Overall, we believe continued strengthening of '0015's efficacy and safety profile could position the program to largely mitigate the IP overhang over time, though additional data will be required to confirm this trajectory.*

Balanced risk-reward, with a defined path to a more constructive stance. While risks remain, including small sample sizes at 32mg in PDAC, immature durability, evolving combination data, and dependence on regulatory alignment and trial execution, we view these as addressable with continued data generation.

Importantly, recent progress across efficacy consistency, emerging combination feasibility, a clearer and accelerated registration strategy, external validation, and a funded development plan incrementally improves the risk-reward. **For us to become more bullish on the stock, we would look for several key gating items:** (1) longer duration of follow-up to establish durability, (2) a larger and more representative patient dataset to confirm consistency of response, and (3) continued execution on trial initiation and enrollment in line with the outlined registration path. Delivery on these elements, particularly more mature data and progression into registration-directed studies, could support an increase in probability of success in NSCLC and PDAC from 50% toward 60%, more consistent with our thresholds for registrational-stage programs, and implying ~20% upside to our price target.

Model updates: We raise our PoS for ERAS-0015/ERAS-4001 in PDAC to 50% (from 40%), NSCLC to 50% (from 35%) and CRC to 45% (from 35%), reflecting increased confidence, accelerated development path and a more validated RAS-targeting landscape. We pull forward our assumed NSCLC launches by one year to 2030/2031, now model simultaneous 1L/2L PDAC launches in 2030, raise our gross price assumption to \$40k (from \$30k) to reflect premium positioning, and adjust our COGS, SG&A, and R&D expenses accordingly. For '0015/'4001, we now model ~\$3.6bn unadj. peak sales (~\$1.8bn risk-adj.) vs. Street's ~\$4bn unadj. (~\$1.6bn risk-adj.) in PDAC, ~\$2.4bn unadj. peak sales (~\$1.2bn risk-adj.) vs. Street's ~\$2.3bn unadj. (~\$800mn risk-adj.) in NSCLC, and ~\$920mn unadj. peak sales (~\$400mn risk-adj.) vs. Street's ~\$1.7bn unadj. (~\$630mn risk-adj.) in CRC. We also incorporate the recent upsized public offering, including the incremental cash and associated dilution. The net result is that our PT goes to \$21 (from \$12).

Related Research:

- [Biotechnology: RAS Oncology Toolkit for ASCO 2026: What You Need to Know \(29 May 2026\)](#)
- [Erasca, Inc.: Assessing the Patent Infringement Risk of ERAS-0015 \(26 May 2026\)](#)
- [Biotechnology: Takeaways From Our KOL Call: Recent RAS Data Updates \(17 May 2026\)](#)
- [Erasca, Inc.: Preliminary Phase 1 Data ERAS-0015 Could Appear Compelling – Waiting to Mature \(28 Apr 2026\)](#)
- [Biotechnology: Takeaways From Our KOL Call: RAS-Targeting Agents \(2 Mar 2026\)](#)
- [Biotechnology: Takeaways From Our KOL Call: The RAS-Targeting Landscape \(20 Feb 2026\)](#)

Risk Reward – Erasca, Inc. (ERAS.O)

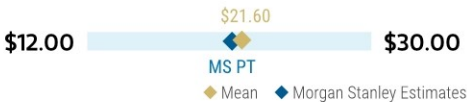
ERAS-0015 and ERAS-4001 Drive Risk/Reward

PRICE TARGET \$21.00

Our PT is derived from a discounted cash flow analysis that assumes a 15% discount rate and 2.5% terminal growth rate.

Consensus Price Target Distribution

Source: Refinitiv, Morgan Stanley Research



RISK REWARD CHART



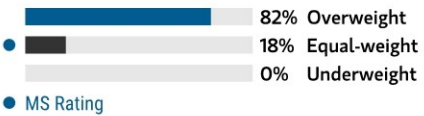
Key: — Historical Stock Performance ● Current Stock Price ◆ Price Target

Source: Refinitiv, Morgan Stanley Research

EQUAL-WEIGHT THESIS

We are encouraged by ERAS-0015's emerging clinical profile, potential combination opportunities, and opportunity for an accelerated registration path, but look for further validation in upcoming updates with a larger patient dataset. We also look for updates on ERAS-4001 and potential partnership opportunities for naporafenib. We think ERAS shares may continue trade at a premium to fair value as the company's assets ("RAS oncology") hold potential strategic value.

Consensus Rating Distribution



Source: Refinitiv, Morgan Stanley Research

Risk Reward Themes

Disruption: Positive

View descriptions of Risk Rewards Themes [here](#)

BULL CASE

\$32.00

DCF

Better-than-expected data are reported for ERAS-0015 and ERAS-4001, resulting in higher PoS and peak market shares. We model higher PoS for naporafenib.

BASE CASE

\$21.00

DCF

ERAS-0015 and ERAS-4001 data support continued advancement in RASm solid tumors and KRASm solid tumors. We model a naporafenib profit share.

BEAR CASE

\$2.00

DCF

Disappointing data and updates are reported limiting commercialization. ERAS shares trade below cash.

Risk Reward – Erasca, Inc. (ERAS.O)

KEY EARNINGS INPUTS

Drivers	Dec 2025	Dec 2026e	Dec 2027e	Dec 2028e
Total Revenues (\$, mm)	0	0	0	0
FCF (\$, mm)	(120)	(147)	(158)	(189)
Peak Revenues (\$, mm)	2,978	2,978	2,978	2,978

CATALYST CALENDAR

Date	Event	Source: Refinitiv, Morgan Stanley
H2 2026	ERAS-4001 Phase 1 BOREALIS-1 monotherapy data in KRASm solid tumors	
H1 2027	ERAS-0015 monotherapy expansion data	
H1 2027	ERAS-0015 combination dose escalation data	

INVESTMENT DRIVERS

- ERAS-0015 Phase 1 AURORAS-1 data
- ERAS-4001 Phase 1 BOREALIS-1 data

GLOBAL REVENUE EXPOSURE



Source: Morgan Stanley Research Estimate
View explanation of regional hierarchies [here](#)

MS ALPHA MODELS

5/5 BEST	24 Month Horizon	5/5 MOST	3 Month Horizon
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Source: Refinitiv, FactSet, Morgan Stanley Research; 1 is the highest favored Quintile and 5 is the least favored Quintile

RISKS TO PT/RATING

RISKS TO UPSIDE

- Positive ERAS-0015 and ERAS-4001 clinical data
- Faster than expected regulatory path
- Encouraging naporafenib partnering updates

RISKS TO DOWNSIDE

- Disappointing ERAS-0015 and ERAS-4001 clinical data
- Slower than expected regulatory path
- Disappointing naporafenib partnering updates

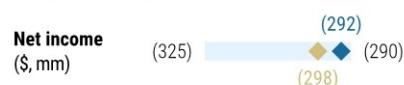
OWNERSHIP POSITIONING

Inst. Owners, % Active	75.5%	
HF Sector Long/Short Ratio	2.2x	
HF Sector Net Exposure	17.5%	

Refinitiv; MSPB Content. Includes certain hedge fund exposures held with MSPB. Information may be inconsistent with or may not reflect broader market trends. Long/Short Ratio = Long Exposure / Short exposure. Sector % of Total Net Exposure = (For a particular sector: Long Exposure - Short Exposure) / (Across all sectors: Long Exposure - Short Exposure).

MS ESTIMATES VS. CONSENSUS

FY Dec 2026e



◆ Mean ◆ Morgan Stanley Estimates

Source: Refinitiv, Morgan Stanley Research

Risk Reward Reference links

1. View explanation of Options Probabilities methodology - [Options_Probabilities_Exhibit_Link.pdf](#)
2. View descriptions of Risk Rewards Themes - [RR_Themes_Exhibit_Link.pdf](#)
3. View explanation of regional hierarchies - [GEG_Exhibit_Link.pdf](#)
4. View explanation of Theme/Exposure methodology - [ESG_Sustainable_Solutions_External_Link.pdf](#)
5. View explanation of HERS methodology - [ESG_HERS_External_Link.pdf](#)

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Therapeutics Plc, Structure Therapeutics Inc, Tenaya Therapeutics Inc, Trevi Therapeutics Inc, Tscan Therapeutics Inc, Ultragenyx Pharmaceutical Inc, Vertex Pharmaceuticals, Viking Therapeutics Inc, Vir Biotechnology Inc, Zenas Biopharma Inc, Zentalis Pharmaceuticals Inc.

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Global Stock Ratings Distribution

(as of June 30, 2026)

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Stock Rating Category	Coverage Universe		Investment Banking Clients (IBC)			Other Material Investment Services Clients (MISC)	
	Count	% of Total	Count	% of Total IBC	% of Rating Category	Count	% of Total Other MISC
Overweight/Buy	1544	42%	453	49%	29%	757	44%
Equal-weight/Hold	1577	43%	390	42%	25%	769	44%
Not-Rated/Hold	3	0%	1	0%	33%	1	0%
Underweight/Sell	544	15%	89	10%	16%	204	12%
Total	3,668		933			1731	

Data include common stock and ADRs currently assigned ratings. Investment Banking Clients are companies from whom Morgan Stanley received investment banking compensation in the last 12 months. Due to rounding off of decimals, the percentages provided in the "% of total" column may not add up to exactly 100 percent.

Analyst Stock Ratings

Overweight (O). The stock's total return is expected to exceed the average total return of the analyst's industry (or industry team's) coverage universe, on a risk-adjusted basis, over the next 12-18 months.

Equal-weight (E). The stock's total return is expected to be in line with the average total return of the analyst's industry (or industry team's) coverage universe, on a risk-adjusted basis, over the next 12-18 months.

Not-Rated (NR). Currently the analyst does not have adequate conviction about the stock's total return relative to the average total return of the analyst's industry (or industry team's) coverage universe, on a risk-adjusted basis, over the next 12-18 months.

Underweight (U). The stock's total return is expected to be below the average total return of the analyst's industry (or industry team's) coverage universe, on a risk-adjusted basis, over the next 12-18 months.

Unless otherwise specified, the time frame for price targets included in Morgan Stanley Research is 12 to 18 months.

Analyst Industry Views

Attractive (A): The analyst expects the performance of his or her industry coverage universe over the next 12-18 months to be attractive vs. the relevant broad market benchmark, as indicated below.

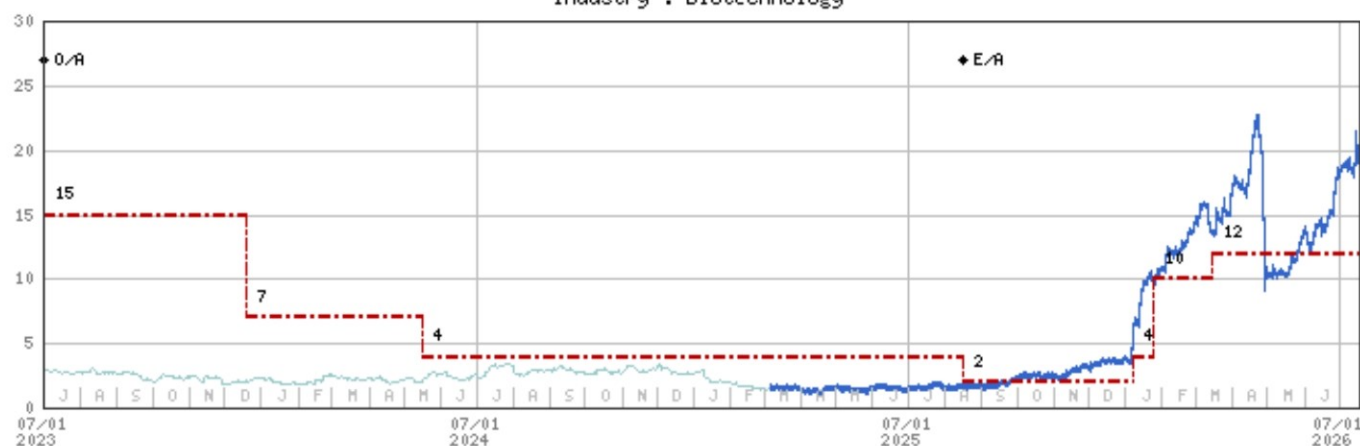
In-Line (I): The analyst expects the performance of his or her industry coverage universe over the next 12-18 months to be in line with the relevant broad market benchmark, as indicated below.

Cautious (C): The analyst views the performance of his or her industry coverage universe over the next 12-18 months with caution vs. the relevant broad market benchmark, as indicated below.

Benchmarks for each region are as follows: North America - S&P 500; Latin America - relevant MSCI country index or MSCI Latin America Index; Europe - MSCI Europe; Japan - TOPIX; Asia - relevant MSCI country index or MSCI sub-regional index or MSCI AC Asia Pacific ex Japan Index.

Stock Price, Price Target and Rating History (See Rating Definitions)

Erasca, Inc. (ERAS.O) - As of 07/17/26 GMT in USD
Industry : Biotechnology



Stock Rating History: 8/10/21 : E/I; 11/5/22 : E/A; 2/3/23 : 0/A; 8/17/25 : E/A

Price Target History: 8/10/21 : 25; 1/31/22 : 19; 7/14/22 : 15; 12/19/23 : 7; 5/17/24 : 4; 8/17/25 : 2; 1/8/26 : 4; 1/25/26 : 10; 3/15/26 : 12

Source: Morgan Stanley Research

Date Format : MM/DD/YY

Price Target --

No Price Target Assigned (NA)

Stock Price (Not Covered by Current Analyst) — Stock Price (Covered by Current Analyst) —

Stock and Industry Ratings (abbreviations below) appear as ♦ Stock Rating/Industry View

Stock Ratings: Overweight (O) Equal-weight (E) Underweight (U) Not-Rated (NR) No Rating Available (NA)

Industry View: Attractive (A) In-line (I) Cautious (C) No Rating (NR)

Effective January 13, 2014, the stocks covered by Morgan Stanley Asia Pacific will be rated relative to the analyst's industry (or industry team's) coverage.

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INDUSTRY COVERAGE: Biotechnology

COMPANY (TICKER)	RATING (AS OF)	PRICE* (07/16/2026)
Judah C. Frommer, CFA		
4D Molecular Therapeutics Inc (FDMT.O)	E (11/06/2025)	\$10.84
Abivax (ABVX.O)	O (07/23/2025)	\$137.77
AgomAb Therapeutics NV (AGMB.O)	O (03/03/2026)	\$12.32
Arcutis Biotherapeutics, Inc. (ARQT.O)	O (07/02/2025)	\$27.74
Avalyn Pharma Inc (AVLN.O)	O (05/25/2026)	\$25.68
Belite Bio Inc (BLTE.O)	O (01/06/2026)	\$150.67
Bicara Therapeutics (BCAX.O)	O (10/08/2024)	\$27.60
Celldex Therapeutics Inc (CLDX.O)	O (03/20/2025)	\$35.40
Compass Pathways PLC (CMPS.O)	O (08/08/2025)	\$12.43
enGene Holdings Inc. (ENGN.O)	U (06/18/2026)	\$1.92
Evomune Inc (EVMN.N)	O (12/01/2025)	\$12.14
Genmab A/S (GMAB.O)	E (02/16/2026)	\$28.98
Immix Biopharma (IMMX.O)	O (03/25/2026)	\$10.06
Incyte Corp (INCY.O)	E (07/02/2025)	\$116.65
Kalaris Therapeutics (KLRS.O)	O (04/16/2026)	\$4.61
Kymera Therapeutics Inc (KYMR.O)	O (07/02/2025)	\$113.99
Lakefront Biotherapeutics (LKFT.O)	E (07/10/2026)	\$29.60
ProKidney Corp (PROK.O)	E (11/10/2022)	\$1.61
PTC Therapeutics (PTCT.O)	O (03/06/2025)	\$78.52
Regenxbio Inc (RGNX.O)	O (11/15/2024)	\$11.20
Trevi Therapeutics Inc (TRVI.O)	O (08/21/2025)	\$17.36
Zenas Biopharma Inc (ZBIO.O)	E (01/05/2026)	\$28.24

Maxwell Skor

Adagene Inc. (ADAG.O)	E (01/30/2025)	\$3.58
Ascendis Pharma A/S (ASND.O)	O (05/05/2025)	\$259.80
Atea Pharmaceuticals Inc (AVIR.O)	E (08/12/2024)	\$4.72
Bicycle Therapeutics Plc (BCYC.O)	E (02/14/2022)	\$4.18
Crinetics Pharmaceuticals (CRNX.O)	++	\$83.91
CRISPR Therapeutics AG (CRSP.O)	E (06/11/2026)	\$48.57
Cytokinetics Inc (CYTK.O)	O (02/13/2025)	\$80.36
Insmmed Inc (INSM.O)	O (03/30/2026)	\$107.46
Monopar Therapeutics (MNPR.O)	O (01/09/2026)	\$108.52
Pharvaris N.V. (PHVS.O)	O (08/15/2023)	\$33.66
Sana Biotechnology, Inc (SANA.O)	O (03/01/2021)	\$3.17
Tscan Therapeutics Inc (TCRX.O)	E (11/13/2025)	\$0.90
Ultragenyx Pharmaceutical Inc (RARE.O)	O (03/27/2019)	\$30.22

Michael E Ulz

Aardvark Therapeutics Inc. (AARD.O)	U (05/15/2026)	\$6.33
Alnylam Pharmaceuticals Inc (ALNY.O)	E (09/09/2022)	\$283.15
Arrowhead Pharmaceuticals Inc (ARWR.O)	O (04/21/2026)	\$71.38
BioAge Labs Inc (BIOA.O)	E (12/05/2025)	\$22.16
Caballetta Bio Inc (CABA.O)	O (01/27/2023)	\$2.67
Dyne Therapeutics Inc (DYN.O)	O (04/30/2024)	\$22.72
Fractyl Health Inc (GUTS.O)	E (01/29/2026)	\$0.85
Ionis Pharmaceuticals Inc (IONS.O)	O (07/30/2025)	\$54.65
Kyverna Therapeutics (KYTX.O)	O (03/04/2024)	\$7.44
Mirum Pharmaceuticals (MIRM.O)	O (11/13/2023)	\$117.36
Rhythm Pharmaceuticals Inc (RYTM.O)	O (12/19/2023)	\$106.09
Rocket Pharmaceuticals Inc (RCKT.O)	E (05/27/2025)	\$3.41
Sarepta Therapeutics Inc (SRPT.O)	E (06/16/2025)	\$17.23
Silence Therapeutics Plc (SLN.O)	O (05/08/2023)	\$10.92
Tenaya Therapeutics Inc (TNYA.O)	O (08/24/2021)	\$0.84
Viking Therapeutics Inc (VKTX.O)	O (06/27/2024)	\$36.37
Vir Biotechnology Inc (VIR.O)	O (01/08/2025)	\$9.40
Zentalis Pharmaceuticals Inc (ZNTL.O)	E (06/18/2024)	\$4.57

Sean Laaman, Ph.D.

ABSCI CORP (ABSI.O)	E (01/08/2026)	\$8.23
Acadia Pharmaceuticals Inc (ACAD.O)	E (08/06/2024)	\$25.63
Alector Inc (ALEC.O)	U (11/26/2024)	\$1.47
argenx SE (ARGX.O)	O (07/02/2025)	\$857.97
Axsome Therapeutics (AXSM.O)	E (01/08/2026)	\$237.04
BeOne Medicines (ONC.O)	O (12/02/2024)	\$317.42
Biomarin Pharmaceutical Inc (BMRN.O)	O (04/27/2026)	\$60.00
BridgeBio Oncology Therapeutics (BBOT.O)	O (12/05/2025)	\$8.31
BridgeBio Pharma Inc (BBIO.O)	O (01/06/2026)	\$80.30
CG Oncology (CGON.O)	O (02/19/2024)	\$69.50
Contineum Therapeutics (CTNM.O)	E (01/08/2026)	\$14.01
Cullinan Therapeutics (CGEM.O)	O (03/06/2025)	\$16.41
Denali Therapeutics Inc (DNLI.O)	O (03/06/2025)	\$22.73
Disc Medicine Inc (IRON.O)	O (11/04/2024)	\$73.47
Eikon Therapeutics Inc (EIKN.O)	O (03/02/2026)	\$9.95
Erasca, Inc. (ERAS.O)	E (08/17/2025)	\$19.02
Exelixis Inc. (EXEL.O)	E (01/08/2026)	\$55.68
Generate Biomedicines Inc (GENB.O)	O (03/24/2026)	\$13.66
Halozyne Therapeutics, Inc (HALO.O)	O (08/05/2025)	\$77.54
Immunocore Holdings Ltd (IMCR.O)	E (12/12/2024)	\$34.47
MapLight Therapeutics (MPLT.O)	O (11/21/2025)	\$37.22

Neurocrine Biosciences Inc (NBIX.O)	E (01/08/2026)	\$171.55
Recursion Pharmaceuticals Inc (RXRX.O)	E (05/22/2023)	\$3.02
Schrodinger Inc. (SDGR.O)	E (11/19/2021)	\$15.60
Terence C Flynn, Ph.D.		
Alumis Inc. (ALMS.O)	O (05/30/2025)	\$27.23
Amgen Inc. (AMGN.O)	E (10/16/2023)	\$371.58
Arcus Biosciences Inc. (RCUS.N)	E (01/08/2026)	\$27.37
Arvinas Inc (ARVN.O)	E (04/06/2022)	\$8.03
Biogen Inc (BIIB.O)	E (10/31/2024)	\$208.92
Biohaven Ltd (BHV.N)	O (07/24/2024)	\$15.44
BioNTech SE (BNTX.O)	O (09/23/2024)	\$91.56
Gilead Sciences Inc. (GILD.O)	O (01/10/2025)	\$136.30
Intellia Therapeutics Inc (NTLA.O)	E (01/26/2025)	\$11.86
Legend Biotech Corp (LEGN.O)	O (01/31/2022)	\$22.71
Moderna Inc (MRNA.O)	E (12/16/2020)	\$63.15
Nurix Therapeutics Inc. (NRIX.O)	O (01/08/2026)	\$23.07
Prime Medicine Inc (PRME.O)	E (11/14/2022)	\$3.22
Regeneron Pharmaceuticals Inc. (REGN.O)	E (12/03/2025)	\$678.94
Structure Therapeutics Inc (GPCR.O)	O (09/22/2024)	\$46.03
United Therapeutics Corp (UTHR.O)	E (07/11/2024)	\$529.05
Vertex Pharmaceuticals (VRTX.O)	++	\$486.03

Stock Ratings are subject to change. Please see latest research for each company.

* Historical prices are not split adjusted.